

Surgery and pregnancy

Most operations are safe during pregnancy. Some are done under local anaesthetic, but others may involve having a general anaesthetic.

On this page

- [When you have surgery for cancer during pregnancy](#)
- [Possible complications](#)
- [Blood clots](#)
- [Breast cancer and surgery during pregnancy](#)
- [Cancer of the cervix and surgery during pregnancy](#)
- [Melanoma and surgery during pregnancy](#)
- [How we can help](#)

Created in partnership with [Mummy's Star](#)

When you have surgery for cancer during pregnancy

Most operations are safe during pregnancy. But it depends on what [type of surgery](#) you need and [how many weeks pregnant you are](#). Some operations use a local anaesthetic. Others may use a general anaesthetic. A regional anaesthetic, such as an epidural, numbs an area of the body. You may have this instead of a general anaesthetic.

Your cancer doctor and pregnancy doctor (obstetrician) will decide the best time for you to have surgery. They may recommend delaying it until you are in your [second trimester](#). This is because having a general anaesthetic in the first trimester can slightly increase the risk of miscarriage. Surgery during the [second or third trimester](#) can increase the risk of going into labour early or having a late miscarriage. You may be offered drugs to reduce the risk of you having contractions and going into labour.

Your obstetrician and an anaesthetist will talk with you about the operation. They will explain how they check on you and the baby during surgery. Your obstetrician may want to check the baby's heart rate before and after surgery.

Possible complications

The risks and complications depend on the [type of surgery](#) you are having. Your surgeon will talk to you about them.

Surgery to your abdomen (tummy area) or your pelvis may have more risk of complications. This is because the area is so close to the womb and baby. There is more risk if you are more than 25 weeks pregnant.

If you need this type of surgery, you and the baby will be checked very closely during the operation. If you are later in your pregnancy, the obstetrician may be there. This is to make sure there is expert help available if there are any problems with the baby during surgery.

Blood clots

Pregnancy and surgery both increase the risk of a [blood clot](#). Cancer itself can also increase the risk of a blood clot. Your surgeon and specialist nurse will give you advice about ways to reduce this risk.

They may ask you to wear compression stockings before your operation and for a few weeks after it. They will also encourage you to get up and walk about soon after your operation. You may have injections under the skin to help reduce the risk of getting a clot.

A clot can cause:

- pain, or warmth and swelling in your leg or arm
- breathlessness
- pain in your chest.

Contact your doctor straight away if you have any of these symptoms. A blood clot is serious. But doctors can treat it with drugs that thin the blood.

Breast cancer and surgery during pregnancy

If you have [breast cancer](#), you are usually given a choice of operations. This is the same for people who are not pregnant. You may have surgery to remove lymph nodes under your arm at the same time as breast surgery.

Your surgeon and breast care nurse will talk to you about your options and the best timing for [surgery](#).

They may ask you to decide whether you want them to:

Remove only the cancer (breast-conserving surgery)

You may have surgery to remove only the area of cancer. This is called [breast-conserving surgery](#). You will need [radiotherapy](#) to the breast afterwards. Radiotherapy reduces the risk of the cancer coming back in the breast.

You will not have radiotherapy while you are pregnant. Your doctors will tell you if having a delay between surgery and radiotherapy is safe for you. This depends on how many weeks pregnant you are. If [chemotherapy](#) is part of your treatment plan, you will have this before radiotherapy.

Remove the whole breast (mastectomy)

Sometimes the surgeon may recommend removing the whole breast. This is called a [mastectomy](#). You may need this surgery because the lump is too large or because there is cancer in different parts of the breast.

It is safe to have a mastectomy during pregnancy. If you want [breast reconstruction](#), you can usually have this after the baby is born and your cancer treatment has finished.

You may have [chemotherapy](#) to shrink the cancer before surgery. This means you may not need a mastectomy. You may have [breast-conserving surgery](#) after the baby is born.

Sentinel lymph node biopsy (SLNB)

Sometimes your cancer doctor will offer you a test called a [sentinel lymph node biopsy \(SLNB\)](#). You may have an SLNB during your operation. Your surgeon will explain if it is suitable for you.

An SLNB usually checks about 1 to 3 lymph nodes in the armpit to see if they contain cancer cells. If there are no cancer cells, you will not need further surgery to remove more lymph nodes.

For the test, a small amount of radioactive liquid is injected into your breast. There is no evidence that this is harmful to the baby. Sometimes a blue dye is injected into the breast. This stains the nearby lymph nodes. The dye is not usually given during pregnancy.

Cancer of the cervix and surgery during pregnancy

Surgery for [cervical cancer](#) when you are pregnant will depend on:

- [the stage of the cancer](#)
- [how many weeks pregnant you are](#)
- [your choices](#).

Normally 1 of the main treatments for cervical cancer is to remove the womb. This is called a [hysterectomy](#). But other types of surgery and options are sometimes used. These may be possible during pregnancy.

Monitoring early stage cancer of the cervix

Many cervical cancers diagnosed during pregnancy are at an [early stage](#). If the cancer is in the very early stages, it may be possible to delay surgery and [monitor the cancer](#) until the baby is born. If there are signs the cancer is growing, your doctors can give you [chemotherapy](#).

Delaying surgery

If you are in early pregnancy, your doctors cannot be sure how delaying surgery may affect the cancer. Sometimes they may advise [ending the pregnancy](#) so you can have a hysterectomy. This is a hard decision to make, especially as the surgery means you can no longer get pregnant. Your doctors and nurses will give you a lot of support. It is important to talk to your doctors about any fertility worries you may have.

Your doctors will explain the risks of continuing with the pregnancy.

In some situations, you may have chemotherapy so that surgery can be delayed until the baby has grown enough to be born. You can then have the surgery after the baby is born. You will usually have a caesarean section (C-section) to deliver the baby. Surgery to remove the cancer can be done at the same time. Chemotherapy can only be given after you are 14 weeks pregnant. You may need more chemotherapy and radiotherapy after the birth.

Surgery to remove the pelvic lymph nodes

Your doctors may recommend an operation to remove the lymph nodes (glands) in your pelvis if:

- your cancer is at an early stage
- you are under 22 weeks pregnant.

This will check if the nodes contain any cancer cells. Your surgeon may advise this so they can be certain the cancer is at an [early stage](#).

If there are cancer cells in the lymph nodes you may be offered further treatment. Your doctors and nurses will talk about this with you and give you a lot of support.

The operation is done under a general anaesthetic using keyhole surgery (laparoscopically). The risk of complications or bleeding may be slightly higher when you are pregnant. Your doctors and nurses will monitor you closely. This means they can treat you quickly if any complications develop.

Trachelectomy

A [trachelectomy](#) removes most of the cervix and the upper part of the vagina. If the cancer is very small and at an early stage, it may be possible to do the surgery during pregnancy. This may happen if cancer is diagnosed early in the pregnancy and you want to continue with the pregnancy. Your doctors will usually advise removing the pelvic lymph nodes first. This is to check the cancer is at an [early stage](#).

Sometimes the surgeon will put a stitch at the base of womb during the trachelectomy. This stitch helps keep the womb closed and reduces the risk of early birth.

Trachelectomy risks include bleeding and losing the baby after the operation. Doctors will talk to you to make sure you fully understand the risks involved and any other options.

Having a trachelectomy during pregnancy is very rare. But some who do have it give birth to healthy babies. It is very specialised surgery. It is only done in certain hospitals by surgeons who are experts in this area.

Hysterectomy after the birth

You may need an operation to [remove the womb \(hysterectomy\)](#) after the birth. This may be at the same time as the C-section. Doctors do not advise a vaginal delivery, as there are possible risks of bleeding from the cancer.

An obstetrician will deliver your baby through a cut made in your tummy (C-section). A gynaecological cancer surgeon will do the hysterectomy.

Your doctors and nurses will talk with you before surgery so you understand what will happen.

Melanoma and surgery during pregnancy

Surgery is the main treatment for [melanoma](#). Early-stage melanomas are usually cured with [surgery](#). This surgery is safe during pregnancy. It is important not to delay surgery because you are pregnant. You can usually have the melanoma removed using a local anaesthetic.

Sentinel lymph node biopsy (SLNB)

Sometimes your cancer doctor will offer you a test called a [sentinel lymph node biopsy \(SLNB\)](#). If you have early melanoma, you will not usually need this. An SLNB is not a treatment, but it can tell you and your doctors more about the stage of the melanoma.

An SLNB checks if any melanoma cells have spread to nearby lymph nodes (glands). This is the most common place melanoma can spread to. You may have the test to see if you need treatment after surgery, to try to reduce the risk of melanoma coming back.

If you need further treatment after SLNB, you will usually have [immunotherapy or targeted therapy drugs](#). You cannot have these drugs during pregnancy. But you must start them within a set period of time after the SLNB. This means you will have the SLNB in the [third trimester](#) of your pregnancy or after the baby has been born. The drugs are as effective at reducing the risk of melanoma coming back when they are given in this way.

An SLNB removes the first lymph node or nodes called the sentinel nodes. You need a general anaesthetic to have it done. The doctor injects a small amount of radioactive liquid close to the lymph nodes. There is no evidence that this is harmful for the baby. Usually, you would also have a blue dye injected into the breast to stain the nearby lymph nodes. But this dye is not usually given during pregnancy.

Advanced melanoma

If [melanoma has spread](#) to nearby lymph nodes or areas of skin, or to other parts of the body, your doctor may talk to you about targeted and immunotherapy drugs. Depending on the stage of the melanoma and how many weeks pregnant you are, your cancer team may talk to you about [having the baby delivered early](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our pregnancy and cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

de Haan J, Verheecke M, et al.

[Oncological management and obstetric and neonatal outcomes for women diagnosed with cancer during pregnancy: a 20-year international cohort study of 1170 patients](#)

. Lancet Oncology. 2018. Vol 19 [accessed October 2023]

Public Health England, Cancer before, during and after pregnancy, National Cancer Registration and Analysis Service 2018 [accessed October 2023]

Silverstein J, Post AL, Chien AJ, Olin R, Tsai KK, Ngo Z, Van Loon K.

[Multidisciplinary management of cancer during pregnancy](#). JCO Oncol Pract. 2020 Sep;16(9):545-557.

doi: 10.1200/OP.20.00077. PMID: 32910882. [accessed September 2023]

Wolters V, Heimovaara J, Maggen C, et al. [Management of pregnancy in women with cancer](#). Int J Gynecol Cancer 2021;31:314–322.

[Cancer Information Development team](#)

Author

[Dr Rebecca Roylance](#)

Reviewer

Consultant Medical Oncologist & Honorary Associate Professor

[University College Hospitals, London](#)

Date reviewed

Reviewed: 01 March 2025 | Next review: 01 March 2028

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help